

May 21, 2026

Ventures Team
25Madison

Re: Entrepreneur-in-Residence, AI RCM Platform

Most EIR candidates bring either domain expertise or technical capability. I bring both—because I've already done what this role exists to create. I am the **Founder and Chief Architect of Denial Doctor**, a HIPAA-compliant, multi-agent AI platform that I conceived, designed, and shipped into production for the healthcare revenue cycle. Six specialized AI agents handle denial classification, appeal drafting, payer follow-up orchestration, predictive scoring, root-cause analytics, and compliance monitoring—delivering a **320% ROI and \$12 million in annualized savings**. I didn't advise someone else to build it. I didn't write a white paper about it. I built it, deployed it, and proved it works with real money on the line. That is the operator-builder profile this role demands, and I live it every day.

My 16 years as **Director of Revenue Cycle Operations at Dharma Solutions** gave me the domain authority that pure technologists lack. I manage **120+ FTEs**, oversee **\$500 million+ in annual revenue**, and have **recovered over \$47 million in denied revenue** while reducing Days in A/R from 65 to 42. I know where every dollar leaks in the revenue cycle because I've spent 16 years plugging those leaks manually before deciding to build the AI to do it at scale. I understand FHIR R4 integration because my platform lives inside that ecosystem. I understand EDI 837/835 transaction flows because my teams process them daily. I understand payer behavior because I've negotiated with every major commercial and government payer. When I architect an AI solution for RCM, it works because it's grounded in operational truth, not theoretical models.

Here's what I'd build at 25Madison: not another AI copilot that sits on top of an EHR and suggests things nobody acts on, but an **autonomous multi-agent system that executes end-to-end revenue cycle tasks**—from claim submission through denial resolution to payment reconciliation. The market opportunity is massive: U.S. healthcare loses \$262 billion annually to denied claims, and the existing solutions are either rule-based engines that can't adapt or generic LLM wrappers that don't understand payer-specific behavior. I've already proven the multi-agent architecture works. I've already validated the ROI. The question isn't whether AI can transform RCM—it's who can build and scale it fastest. With 25Madison's venture infrastructure and my domain-plus-technical depth, we can own this category.

I'm also the author of **"The \$50K Leak,"** published on Amazon Kindle, which dissects the systemic revenue losses in healthcare billing and prescribes actionable fixes. Writing that book forced me to crystallize the problem thesis, and building Denial Doctor was the natural execution of that thesis. At 25Madison, I'd bring the same clarity of vision and velocity of execution. I'm not looking for a sandbox to experiment in. I'm looking for the capital, network, and strategic partnership to scale what I've already validated. If that's the kind of EIR you want, we should talk.

My full journey—from RCM operator to AI platform founder to published author—is at rajeshkantubhukta.github.io. Let's build something that matters.

Rajesh Kantubhukta

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